CROSS-ADDICTION 2026

FACILITATOR GUIDE

Clinical teaching notes for the locked 34-slide patient-education deck

Core design rule: The slides are patient signposts and facilitator cues—not a transcript. This guide holds the curriculum that does not belong on the screen.

Primary audience	Patients in SUD treatment / recovery-oriented psychoeducation
Primary clinical frame	DSM-5-TR terminology; ICD-11 only when a classification difference is directly relevant
Facilitation stance	Interactive, de-shaming, clinically precise, misconception-first
Evidence basis	Cross-Addiction 2026 Evidence Audit, September 17, 2026

Prepared as the facilitator companion to Stephanie's Cross-Addiction 2026 redesign

How to Use This Guide

This guide is designed for a live clinical group. It preserves the deck's question → patient thinking → correction → application rhythm. The facilitator does not need to read the guide aloud, cover every optional probe, or turn every note into a mini-lecture.

- Ask before telling. Many slides intentionally surface a misconception before the correction appears.
- Let patients generate examples. Use the listed probes only when discussion stalls or needs redirection.
- Do not diagnose from the case vignettes. The task is to notice patterns, ask better questions, and identify recovery or safety concerns.
- Use humor to lower defensiveness—not to make pain, trauma, sexuality, medication treatment, withdrawal, or shame-sensitive disclosure into the joke.
- The Four Fs are an original reflection framework: Function, Freedom, Focus, Fallout. They are not a diagnostic test and must never be scored or given cutoffs.
- When a safety boundary appears in this guide, it is not optional.

Standing rule: DO NOT TURN STEPHANIE'S TALKING POINTS INTO SCREEN TEXT JUST BECAUSE FRANK FINALLY DISCOVERED THEY EXIST.

Clinical Language Quick Reference

Term	Facilitator distinction
Use	Exposure or consumption. Use alone does not establish misuse, dependence, or SUD.
Misuse	Use outside intended, prescribed, or safer parameters. Misuse is not automatically the same as SUD.
Tolerance	A changed response with repeated exposure. It may occur with prescribed medication and does not by itself establish addiction.
Physical dependence	Physiologic adaptation in which stopping or reducing can produce withdrawal. It can occur during appropriate prescribed treatment.
Withdrawal	Symptoms after reduction or cessation. Medical risk differs by substance.
SUD / addiction	A broader pattern involving impaired control, consequences, priority/salience, and related features; not reducible to tolerance or withdrawal alone.
Psychological reliance	Useful descriptive language when a person feels unable to function, sleep, cope, etc. without something, without assuming a formal addiction diagnosis.

Non-Negotiable Safety Boundaries

- Slide 14: correct the “replacing one drug with another” misconception in the same teaching segment. Buprenorphine and methadone are evidence-based OUD treatments; medication treatment is not clinically equivalent to uncontrolled opioid use.
- Slide 29: opioid plus benzodiazepine exposure raises overdose risk. Nonprescribed opioid/benzodiazepine use warrants prompt clinical assessment.
- Abrupt benzodiazepine discontinuation or rapid reduction can cause serious, potentially life-threatening withdrawal. Heavy-alcohol withdrawal can also become medically dangerous.
- Do not reduce OUD care to surviving detox. Withdrawal management and ongoing evidence-based treatment are different things.
- Naloxone/Narcan training remains in the facility’s required DMH health curriculum. In this group, cross-reference that training rather than duplicating it.
- Do not treat legitimate pain, tolerance, physical dependence, medication requests, misuse, and OUD as interchangeable.

1. CROSS-ADDICTION

INSTRUCTIONAL PURPOSE

Open with inquiry rather than definition.

WHAT THE PATIENT SEES

Title + “When the substance changes, does the pattern change?”

FACILITATION FLOW

- Ask the subtitle and let the room answer without correcting immediately.
- Listen for answers about substances, behaviors, rituals, coping, control, or consequences.
- Tell the group you are going to keep returning to the word pattern.

USEFUL PROBES

- What might change when somebody stops one substance?
- What might not change automatically?

BOUNDARY / DO NOT OVERCLAIM

Guardrail: Do not imply that cross-addiction is inevitable or a formal diagnosis.

TRANSITION

Move directly into their definition, not yours.

2. WHAT DO YOU THINK?

INSTRUCTIONAL PURPOSE

Surface the group’s existing definition and misconceptions.

WHAT THE PATIENT SEES

“In your own words, what does cross-addiction mean?”

FACILITATION FLOW

- Let several patients answer.
- Reflect useful pieces without certifying inaccurate terminology.
- Notice whether the room defines it by the object used or by the pattern around it.

USEFUL PROBES

- Does it have to be another drug?
- Does it have to happen at the same time?
- Would doing something often automatically count?

BOUNDARY / DO NOT OVERCLAIM

Guardrail: Do not validate “Addiction Interaction Disorder” or other obsolete pseudo-diagnostic labels.

TRANSITION

Now give them the working definition.

3. CROSS-ADDICTION

INSTRUCTIONAL PURPOSE

Give a usable recovery definition while separating it from diagnosis.

WHAT THE PATIENT SEES

"A shift from one problematic substance or behavior to another." Reveal: "Useful recovery concept ≠ formal diagnosis."

FACILITATION FLOW

- Define it as recovery-oriented language for a possible shift in problematic pattern.
- Emphasize possible—not guaranteed, universal, or mathematically predictable.
- Use the reveal to distinguish a useful concept from a DSM diagnosis.

MUST-LAND TEACHING

Clinical anchor: Cross-addiction is a recovery/research concept, not a DSM-5-TR diagnosis, and there is no defensible universal substitution rate.

BOUNDARY / DO NOT OVERCLAIM

Guardrail: Do not teach substitution as the expected course of recovery. Evidence is heterogeneous, and concurrent improvement can occur.

TRANSITION

If the object can change, what else can change—and what can stay familiar?

4. WHAT CAN CHANGE?

INSTRUCTIONAL PURPOSE

Broaden the lens from substances to patterns and functions.

WHAT THE PATIENT SEES

Substance → Substance; Substance → Behavior; Behavior → Behavior. "What might stay the same?"

FACILITATION FLOW

- Let the group generate examples before offering any.
- Redirect from labels toward function, control, focus, and consequences.
- Plant the idea that the behavior can look different while serving a familiar job.

USEFUL PROBES

- What could stay the same emotionally?
- What could stay the same about secrecy, planning, or consequences?

BOUNDARY / DO NOT OVERCLAIM

Guardrail: Frequent or rewarding behavior is not automatically an addiction.

TRANSITION

Before we talk about what replaces what, ask what the original substance was doing.

5. WHAT DID YOUR SUBSTANCE DO FOR YOU?

INSTRUCTIONAL PURPOSE

Shift from moral judgment to functional analysis.

WHAT THE PATIENT SEES

Escape • Energy • Confidence • Relief • Numbing • Connection • Excitement • Sleep • Reward • Something else?

FACILITATION FLOW

- Ask the question before revealing examples.
- Normalize that substances often worked at something before they became costly.
- Invite multiple functions; avoid forcing one root cause.

USEFUL PROBES

- What did it make easier?
- What did it make quieter?
- What did it give you that you did not know how to get another way?

MUST-LAND TEACHING

Clinical anchor: A function can be real even when the strategy later becomes harmful.

BOUNDARY / DO NOT OVERCLAIM

Guardrail: Self-reported function does not establish a diagnosis or a single cause.

STEPHANIE VOICE / OPTIONAL

This is a good place for warmth and light humor about the brain keeping a strategy that appears to work; the patient is never the punchline.

TRANSITION

Now name the de-shaming premise directly.

6. THE SUBSTANCE WAS A SOLUTION.

INSTRUCTIONAL PURPOSE

De-shame initial use while introducing loss of flexibility and control.

WHAT THE PATIENT SEES

"Until the solution became another problem." / "The solution becomes the problem when it stops being a choice."

FACILITATION FLOW

- Let the first sentence sit.
- Clarify that people generally do not begin use expecting it to dismantle their lives.
- Discuss how repeated reinforcement, biology, context, and vulnerability can narrow behavioral choice over time.

USEFUL PROBES

- When did the thing that helped start costing more than it gave?
- What changed about your freedom to choose?

MUST-LAND TEACHING

Clinical anchor: The line about choice is experiential teaching language, not a diagnostic threshold.

BOUNDARY / DO NOT OVERCLAIM

Guardrail: Avoid both extremes: "you chose all of this" and "you have no agency at all." Exposure, learning, adaptation, context, and vulnerability interact.

TRANSITION

So how does something that works become the thing we reach for automatically?

7. HOW DOES A SOLUTION BECOME A PATTERN?

INSTRUCTIONAL PURPOSE

Teach reinforcement and pattern learning in plain language.

WHAT THE PATIENT SEES

Something works → I remember it → I use it again → it works again → it becomes easier to reach for. Final question about automatic reaching.

FACILITATION FLOW

- Walk through the sequence slowly and invite personal/nonpersonal examples.
- Optional analogy: a computer learns routines or stores a shortcut; repeated success makes the shortcut easier to retrieve.
- Explicitly say the analogy is a teaching model, not literal neuroscience.

USEFUL PROBES

- What kinds of results train us fastest—relief, reward, escape, connection?
- When does a shortcut become too automatic?

MUST-LAND TEACHING

Clinical anchor: Rewarded or relieving responses can become more available and habitual; habit, craving, compulsion, withdrawal, and goal-directed behavior are related but not interchangeable.

BOUNDARY / DO NOT OVERCLAIM

Guardrail: Do not describe the brain as literally being "coded" or infected with a virus. That is analogy only.

STEPHANIE VOICE / OPTIONAL

The computer analogy can earn a smile; do not turn it into a fake neuroscience TED Talk.

TRANSITION

Which brings us to one of recovery's greatest hits: "But it's not my DOC."

8. "BUT IT'S NOT MY DOC."

INSTRUCTIONAL PURPOSE

Surface a common risk-minimizing argument without giving a blanket abstinence rule.

WHAT THE PATIENT SEES

Question-only misconception prompt.

FACILITATION FLOW

- Ask what patients mean when they say this.
- Validate that prior drug history matters.
- Then ask what information "not my DOC" leaves out.

USEFUL PROBES

- Can something be risky even if it was never your favorite?
- Could a prescribed medication be appropriate even if it affects the brain?

MUST-LAND TEACHING

Clinical anchor: Drug of choice is informal recovery language. It is one piece of history, not a complete safety assessment.

BOUNDARY / DO NOT OVERCLAIM

Guardrail: Do not teach that any psychoactive medication exposure equals relapse.

TRANSITION

Build the fuller assessment.

9. WHAT ACTUALLY CHANGES THE RISK?

INSTRUCTIONAL PURPOSE

Replace DOC-only reasoning with multidimensional assessment.

WHAT THE PATIENT SEES

History • Function • Frequency/amount • Control • Consequences • Recovery stability • Medical context • Honesty/secrecy.

FACILITATION FLOW

- Reveal factors progressively.
- Use a medication example to show why medical context matters.
- Treat secrecy as information to explore, not proof of disorder.

USEFUL PROBES

- What would you want to know before deciding whether something is helping or hurting recovery?

MUST-LAND TEACHING

Clinical anchor: The label or substance name never replaces assessment of the whole pattern.

BOUNDARY / DO NOT OVERCLAIM

Guardrail: Medication decisions require individualized risk-benefit review with the prescriber. Secrecy can reflect fear or stigma as well as a harmful pattern.

TRANSITION

Now interrogate another old recovery shortcut: the gateway idea.

10. WHAT'S THE DEAL WITH "GATEWAY" DRUGS?

INSTRUCTIONAL PURPOSE

Surface the causal gateway belief before correcting it.

WHAT THE PATIENT SEES

Question only.

FACILITATION FLOW

- Ask the room what "gateway drug" means to them.
- Separate "often came first" from "caused what came later."
- Do not rush to the correction until the distinction is audible in the room.

USEFUL PROBES

- What usually gets named as a gateway?
- How do we know whether first means caused?

BOUNDARY / DO NOT OVERCLAIM

Guardrail: The quotation marks matter: this is a claim being examined, not endorsed.

TRANSITION

Show the familiar sequence, then challenge the causal leap.

11. "GATEWAY" DRUGS

INSTRUCTIONAL PURPOSE

Teach sequence ≠ causation.

WHAT THE PATIENT SEES

"Soft" → "Hard." Reveal: "Does first mean caused?"

FACILITATION FLOW

- Let the arrow initially do what old teaching usually does.
- Then interrupt the inference: first is not the same as cause.
- Ask what else could explain why one substance tends to appear earlier.

USEFUL PROBES

- What was easier to get first?
- What was socially acceptable first?
- What was around you first?

MUST-LAND TEACHING

Clinical anchor: Order of initiation can be observed without proving that the first substance biologically caused later use.

BOUNDARY / DO NOT OVERCLAIM

Guardrail: Do not teach the arrow as a biological pathway. "Soft" and "hard" are informal, unstable categories.

TRANSITION

Give the room better explanations than a single gateway story.

12. WHAT ELSE COULD EXPLAIN THE ORDER?

INSTRUCTIONAL PURPOSE

Introduce common-liability and context explanations in patient language.

WHAT THE PATIENT SEES

ACCESS • EXPOSURE • ENVIRONMENT • VULNERABILITY.

FACILITATION FLOW

- Reveal one at a time and ask for examples.
- Explain that multiple explanations can coexist.
- Optional verbal additions: developmental, family, social, and shared vulnerability factors.

USEFUL PROBES

- Which of these mattered most in your environment?
- How can availability shape what gets tried first?

MUST-LAND TEACHING

Clinical anchor: Sequence can reflect shared vulnerability and context rather than a simple one-drug-causes-the-next pathway.

BOUNDARY / DO NOT OVERCLAIM

Guardrail: Do not replace one oversimplification with another; no single factor has to explain every person.

TRANSITION

If "gateway" is messy, so are "soft" and "hard."

13. "SOFT" vs. "HARD"

INSTRUCTIONAL PURPOSE

Expose the ambiguity in informal drug labels.

WHAT THE PATIENT SEES

Access? • Legality? • Acceptance? • Risk?

FACILITATION FLOW

- Ask what patients mean when they use each label.
- Point out that people may be talking about legality, availability, potency, route, overdose risk, dependence liability, or perceived harm.
- Emphasize that risk varies by product, dose, route, person, and context.

USEFUL PROBES

- Is legal the same as safe?
- Is prescription the same as harmless?
- Can two products in the same broad category carry different risks?

MUST-LAND TEACHING

Clinical anchor: "Soft" and "hard" are not standardized clinical classifications.

BOUNDARY / DO NOT OVERCLAIM

Guardrail: Illegal does not itself quantify pharmacologic risk; legal or prescribed does not mean harmless.

TRANSITION

This sets up the medication question that needs an immediate correction.

14. "AREN'T YOU JUST REPLACING ONE DRUG WITH ANOTHER?"

INSTRUCTIONAL PURPOSE

Surface and correct the MOUD misconception.

WHAT THE PATIENT SEES

Question only.

FACILITATION FLOW

- Let patients state the argument.
- Ask what would make prescribed treatment clinically different from uncontrolled use.
- Correct the misconception before leaving this segment: treatment purpose, dosing/monitoring, outcomes, control, and medical context matter.

USEFUL PROBES

- If two things act on the same receptor system, does that make their use pattern clinically identical?
- What happens when we judge treatment by the drug name instead of the outcome and pattern?

MUST-LAND TEACHING

Clinical anchor: Buprenorphine and methadone are evidence-based OUD treatments. Physical dependence or receptor activity does not make medication treatment equivalent to uncontrolled opioid use.

BOUNDARY / DO NOT OVERCLAIM

Guardrail: Never frame MOUD as failed sobriety. Acknowledge misuse/diversion risks without using them to erase treatment benefit.

STEPHANIE VOICE / OPTIONAL

Humor can target the oversimplified argument, not people taking MOUD.

TRANSITION

The next slide gives the assessment questions that keep us out of false-equivalence land.

15. SAME THING?

INSTRUCTIONAL PURPOSE

Teach distinctions among use, misuse, dependence, and disorder through pattern assessment.

WHAT THE PATIENT SEES

WHAT is it? WHY am I using it? HOW am I using it? WHAT is happening because of it?

FACILITATION FLOW

- Use examples: prescribed medication as directed; running out early; nonprescribed use; a retail product used for self-treatment.
- Introduce the quick-reference distinctions from the front of this guide.
- Return to control and consequences rather than moral labels.

USEFUL PROBES

- Is the medication doing the job it was prescribed to do?
- Is the person changing dose, source, route, or purpose?
- What is happening because of the pattern?

MUST-LAND TEACHING

Clinical anchor: Appropriate medical use, misuse, tolerance, physical dependence, withdrawal, and SUD are not interchangeable.

BOUNDARY / DO NOT OVERCLAIM

Guardrail: Benign intent does not eliminate risk; risky intent alone does not establish SUD.

TRANSITION

Now widen the same diagnostic caution to behaviors.

16. CAN YOU GET "ADDICTED" TO ANYTHING?

INSTRUCTIONAL PURPOSE

Separate everyday "addicted" language from clinical diagnosis.

WHAT THE PATIENT SEES

Question only; optional: "What do we mean when we say 'addicted'?"

FACILITATION FLOW

- Let the group name ridiculous and serious examples.
- Ask what they mean: liking it, doing it often, missing it, losing control, or suffering consequences.
- Move from word choice to impairment and control.

USEFUL PROBES

- What would make "I'm addicted to coffee/TV/my phone" casual language rather than a clinical claim?

MUST-LAND TEACHING

Clinical anchor: Not every rewarding or repeated activity is an addiction.

BOUNDARY / DO NOT OVERCLAIM

Guardrail: Ordinary enthusiasm, routine, or high frequency is not enough for disorder-level framing.

STEPHANIE VOICE / OPTIONAL

This is a safe place for confetti-level sarcasm about how the internet is apparently "addicted" to everything.

TRANSITION

Frequency is information—but not the whole answer.

17. "I DO IT A LOT."

INSTRUCTIONAL PURPOSE

Teach why frequency alone is insufficient.

WHAT THE PATIENT SEES

Questions about control, function, inability, displacement, and consequences.

FACILITATION FLOW

- Use a neutral high-frequency example first.
- Then contrast with a lower-frequency behavior that still creates meaningful harm.
- Link the questions to the Four Fs without naming them yet if you want the framework reveal to land.

USEFUL PROBES

- Can something happen every day and still be healthy?
- Can something happen less often and still be dangerous?

MUST-LAND TEACHING

Clinical anchor: Impaired control, priority displacement, distress/impairment, and persistence despite harm tell us more than frequency alone.

BOUNDARY / DO NOT OVERCLAIM

Guardrail: High frequency can be nonpathological; lower frequency can still be risky or impairing.

TRANSITION

Now test that logic against behaviors people casually call addictions.

18. SAME LABEL?

INSTRUCTIONAL PURPOSE

Teach diagnostic humility across repetitive behaviors.

WHAT THE PATIENT SEES

Gambling • Gaming • Shopping • Sex • Exercise • Eating • Social Media • Work. “Are all of these clinically ‘addictions’?” → “Not necessarily.”

FACILITATION FLOW

- Ask which ones patients assume are official diagnoses.
- Use DSM-5-TR as the primary frame: gambling disorder is formally classified as a behavioral addiction; Internet Gaming Disorder is listed for further study.
- If useful, briefly note that ICD-11 differs and formally recognizes gaming disorder. Stop there unless the distinction helps the discussion.

USEFUL PROBES

- Can a behavior be harmful without being called an addiction?
- Could another diagnosis or context better explain the behavior?

MUST-LAND TEACHING

Clinical anchor: The clinical label depends on the classification system and the full assessment—not on how compulsive something looks from across the room.

BOUNDARY / DO NOT OVERCLAIM

Guardrail: Do not call shopping, sex, exercise, eating, social media, or work “official addictions” as a blanket statement. Eating disorders are separate diagnoses; ICD-11 places compulsive sexual behaviour disorder under impulse-control disorders.

STEPHANIE VOICE / OPTIONAL

A little “DSM does not issue a diagnosis every time humans get too enthusiastic about something” energy is acceptable.

TRANSITION

That raises the useful question: do we need the label before we address the harm?

19. DOES THE LABEL HAVE TO COME FIRST?

INSTRUCTIONAL PURPOSE

Give permission to address harm without premature diagnosis.

WHAT THE PATIENT SEES

“A problem is allowed to be a problem before it becomes a diagnosis.”

FACILITATION FLOW

- Ask the question and let the group wrestle with it.
- Emphasize that treatment can address risk, impairment, values conflict, or recovery interference before a diagnostic label is settled.
- Normalize referral/assessment when more information is needed.

USEFUL PROBES

- What could you change today even if nobody has named the pattern yet?

MUST-LAND TEACHING

Clinical anchor: Clinical action can target harm and recovery interference without overdiagnosing.

BOUNDARY / DO NOT OVERCLAIM

Guardrail: Still consider whether another diagnosis, medical issue, or context better explains the behavior.

TRANSITION

Now give them a simple reflection tool for noticing patterns.

20. THE FOUR Fs

INSTRUCTIONAL PURPOSE

Introduce the original reflection framework.

WHAT THE PATIENT SEES

Function • Freedom • Focus • Fallout. Footer: “Reflection tool—not a diagnostic test.”

FACILITATION FLOW

- Name all four once.
- Explain that the tool organizes questions; it does not produce a score or diagnosis.
- Tell patients the next four slides slow each F down.

MUST-LAND TEACHING

Clinical anchor: Function = what it does; Freedom = choice/control; Focus = space/priority; Fallout = cost/consequences.

BOUNDARY / DO NOT OVERCLAIM

Guardrail: Never score the Four Fs, create cutoffs, or imply validation, sensitivity, specificity, or norms.

STEPHANIE VOICE / OPTIONAL

Frank has been explicitly forbidden from turning this into a 47-item inventory. The Ministry thanks you for your cooperation.

TRANSITION

Start with the job the behavior is doing.

21. FUNCTION

INSTRUCTIONAL PURPOSE

Identify reinforcement, coping, and unmet need.

WHAT THE PATIENT SEES

RELIEF • ESCAPE • ENERGY • CONNECTION • REWARD • CONTROL. “What need am I trying to meet?”

FACILITATION FLOW

- Ask patients to distinguish the strategy from the need.
- Validate the need without automatically endorsing the strategy.
- Invite alternative ways the same need might be met later.

USEFUL PROBES

- What do you get immediately?
- What feeling changes?
- What need is underneath it?

MUST-LAND TEACHING

Clinical anchor: A harmful strategy can still be meeting a real need.

BOUNDARY / DO NOT OVERCLAIM

Guardrail: Do not turn “function” into a single-cause explanation.

TRANSITION

Next: how much freedom is left around the strategy?

22. FREEDOM

INSTRUCTIONAL PURPOSE

Explore impaired control and flexibility.

WHAT THE PATIENT SEES

Can I wait? Change the plan? Stop when intended? Keep making rules and breaking them?

FACILITATION FLOW

- Use self-set rules as data, not a morality test.
- Ask about delay, stopping, changing plans, and ability to choose alternatives.
- Notice patterns across time rather than one broken promise.

USEFUL PROBES

- What happens when you decide “not today”?
- Do your rules keep getting rewritten after the fact?

MUST-LAND TEACHING

Clinical anchor: Reduced flexibility and repeated inability to follow intended limits can signal impaired control.

BOUNDARY / DO NOT OVERCLAIM

Guardrail: One broken rule does not diagnose a disorder.

TRANSITION

Then ask how much territory the pattern is occupying.

23. FOCUS

INSTRUCTIONAL PURPOSE

Explore salience, preoccupation, and priority displacement.

WHAT THE PATIENT SEES

THOUGHTS • TIME • PLANNING • MONEY • PRIORITIES. “What is getting pushed aside?”

FACILITATION FLOW

- Ask what the person thinks about before, during, and after.
- Include planning, recovering, hiding, budgeting, arranging, and anticipating—not just the act itself.
- Look for reduced flexibility and displaced priorities.

USEFUL PROBES

- How much of the day is really about the behavior once planning and recovery time count?
- What used to occupy that space?

MUST-LAND TEACHING

Clinical anchor: The relevant issue is not merely intensity of interest; it is how much life reorganizes around the pattern.

BOUNDARY / DO NOT OVERCLAIM

Guardrail: Temporary intense focus can be normative.

TRANSITION

Finally: what is the bill?

24. FALLOUT

INSTRUCTIONAL PURPOSE

Make consequences and values conflict visible.

WHAT THE PATIENT SEES

RELATIONSHIPS • MONEY • HEALTH • WORK • RESPONSIBILITIES • VALUES • RECOVERY.

FACILITATION FLOW

- Ask for both obvious and quiet costs.
- Include internal values conflict, not only external consequences.
- Ask whether the person continues despite seeing the cost.

USEFUL PROBES

- What are you paying for this besides money?
- What matters to you that is getting harder to protect?

MUST-LAND TEACHING

Clinical anchor: Continuing despite harm is an important warning pattern across SUD and recognized behavioral disorders.

BOUNDARY / DO NOT OVERCLAIM

Guardrail: Consequences can occur without addiction; do not reverse-infer a diagnosis.

TRANSITION

Now use the Four Fs on people who are safely fictional.

25. MEET TIM

INSTRUCTIONAL PURPOSE

Apply the framework to relationship/sexual behavior without moralizing.

WHAT THE PATIENT SEES

Early recovery → relationship with group member → meetings become hookups/time together → secrecy → "At least I'm still clean" → missing IOP.

FACILITATION FLOW

- Ask "What do you notice?" before naming anything.
- Work through Function, Freedom, Focus, Fallout.
- Ask "What else would you need to know?" to model diagnostic humility.
- Keep the clinical focus on concealment, boundaries, shifting priorities, and recovery interference.

USEFUL PROBES

- What might the relationship be doing for Tim?
- What changed about his recovery routine?
- What is the difference between "a relationship" and "a pattern that is interfering with treatment"?

MUST-LAND TEACHING

Clinical anchor: The vignette supports discussion of secrecy, rationalization, and treatment disengagement—not a diagnosis of sex or relationship addiction.

BOUNDARY / DO NOT OVERCLAIM

Guardrail: Do not moralize consensual sexuality or relationship status. Humor should not target sex, intimacy, or shame-sensitive disclosure.

TRANSITION

Same framework, different behavior: Lana.

26. MEET LANA

INSTRUCTIONAL PURPOSE

Apply the framework to spending and emotional escape.

WHAT THE PATIENT SEES

90 days sober → more disposable income → shopping as emotional escape → credit-card debt → paycheck disappears → “What’s the harm?”

FACILITATION FLOW

- Ask the group to identify the immediate reward and delayed cost.
- Use the Four Fs.
- Ask what information would distinguish ordinary enjoyment from a concerning pattern.

USEFUL PROBES

- What is shopping doing for her?
- Where is freedom narrowing?
- What is the financial fallout?

MUST-LAND TEACHING

Clinical anchor: Compulsive buying is clinically studied, but this vignette does not establish a standalone DSM-5-TR addiction diagnosis.

BOUNDARY / DO NOT OVERCLAIM

Guardrail: Do not equate spending money or enjoying shopping with addiction.

TRANSITION

Buck gives us a behavior with clearer formal diagnostic recognition.

27. MEET BUCK

INSTRUCTIONAL PURPOSE

Apply the framework to gambling while still avoiding vignette diagnosis.

WHAT THE PATIENT SEES

2+ years sober → casino weekends → unavailable to sponsees → secretly spends half savings → chasing losses.

FACILITATION FLOW

- Ask the group to notice chasing, concealment, preoccupation, financial harm, and role neglect.
- Use the Four Fs.
- Then ask what a full assessment would still need before diagnosis.

USEFUL PROBES

- What is the “one more good weekend” thought doing?
- What priorities changed?
- What is hidden?

MUST-LAND TEACHING

Clinical anchor: Gambling disorder is formally recognized, but diagnosis still requires full assessment, time course, thresholds, and exclusion considerations.

BOUNDARY / DO NOT OVERCLAIM

Guardrail: Do not turn recognizable features into an instant diagnosis.

TRANSITION

Leslie tests whether we can notice harm without inventing a diagnosis.

28. MEET LESLIE

INSTRUCTIONAL PURPOSE

Apply the framework to problematic social-media/online use.

WHAT THE PATIENT SEES

New recovery/loneliness → online connection → increasing time → less available to family → work interference → preoccupation when unable to check.

FACILITATION FLOW

- Start with the legitimate function: connection.
- Use the Four Fs to identify when helpful connection becomes costly or inflexible.
- Ask what else could contribute to the pattern.

USEFUL PROBES

- What is the internet giving Leslie?
- What is getting pushed aside?
- What else would you assess—sleep, mood, anxiety, ADHD, trauma, social context?

MUST-LAND TEACHING

Clinical anchor: Problematic digital/social-media use can cause impairment without "social media addiction" being a formal DSM-5-TR diagnosis.

BOUNDARY / DO NOT OVERCLAIM

Guardrail: Do not diagnose loneliness, high screen time, or frequent phone use.

TRANSITION

Jack brings the same pattern-recognition exercise into a medical and safety context.

29. MEET JACK

INSTRUCTIONAL PURPOSE

Differentiate legitimate treatment from a changing unsafe pattern and land medical safety.

WHAT THE PATIENT SEES

Five years sober → injury → prescribed pain medication → initially appropriate use → runs out early → additional nonprescribed opioids/benzodiazepines → scheming/doctor-shopping.

FACILITATION FLOW

- First ask only: "Where did the pattern change?"
- Protect the first three months from retrospective moralizing: legitimate pain treatment is not the problem by definition.
- Then use the Four Fs and ask what information is still needed.
- After discussion, state the overdose and withdrawal safety boundaries clearly.

USEFUL PROBES

- What changed first—dose, source, control, secrecy, consequences?
- What would you need to know about pain, tolerance, withdrawal, and prescribing?
- What needs prompt medical assessment?

MUST-LAND TEACHING

Clinical anchor: Opioid + benzodiazepine exposure raises overdose risk. Abrupt benzodiazepine or heavy-alcohol cessation can be life-threatening. Withdrawal management is not the same as ongoing OUD treatment. Cross-reference the program's required Narcan training rather than reteaching it here.

BOUNDARY / DO NOT OVERCLAIM

Guardrail: Do not call all pain requests "drug seeking." Legitimate pain, undertreatment, tolerance, physical dependence, misuse, withdrawal, and OUD can overlap and require assessment.

STEPHANIE VOICE / OPTIONAL

No cute line here. This is one of the places where nuance gets the whole stage.

TRANSITION

Now compare the cases without pretending they are clinically identical.

30. WHAT DID THEY HAVE IN COMMON?

INSTRUCTIONAL PURPOSE

Synthesize transdiagnostic warning patterns across different behaviors.

WHAT THE PATIENT SEES

Rationalizing • Preoccupation • Secrecy • Loss of control • Neglected priorities • Continuing despite consequences • Recovery Drift • "Different behavior. Familiar pattern."

FACILITATION FLOW

- Let the group build the list first.
- Reveal only what they miss.
- Define "recovery drift" as descriptive teaching language: movement away from recovery-supportive patterns, not a diagnosis.
- Ask the closing question: "The behavior changed. Did the pattern?"

USEFUL PROBES

- Which warning sign showed up earliest?
- Which ones were present across the most cases?

MUST-LAND TEACHING

Clinical anchor: Shared features can be useful warning signs without making all five behaviors clinically equivalent.

BOUNDARY / DO NOT OVERCLAIM

Guardrail: Secrecy may reflect stigma or fear as well as disorder; explore it rather than treating it as proof.

TRANSITION

Turn the lens inward—but keep disclosure optional.

31. WHAT ABOUT ME?

INSTRUCTIONAL PURPOSE

Move from case analysis to private self-reflection.

WHAT THE PATIENT SEES

“Is there anything I use to change how I feel?” + Four Fs + “Notice the pattern. Don’t rush to label it.”

FACILITATION FLOW

- Give quiet time before asking for any sharing.
- Explicitly say patients may keep their answer private.
- Invite optional sharing about a pattern or need rather than demanding disclosure of a behavior.
- Offer private follow-up if the reflection raises a safety concern.

USEFUL PROBES

- Which F gave you the most useful information?
- What would you want to notice earlier?

MUST-LAND TEACHING

Clinical anchor: The goal is awareness and earlier response, not self-diagnosis.

BOUNDARY / DO NOT OVERCLAIM

Guardrail: Do not pressure disclosure in the group.

TRANSITION

Awareness is useful only if we know what to do with it.

32. PROTECTING RECOVERY

INSTRUCTIONAL PURPOSE

Translate insight into practical recovery-protective actions.

WHAT THE PATIENT SEES

Notice it • Name the need • Talk about it • Add options • Use support.

FACILITATION FLOW

- Connect each action to the Four Fs.
- Emphasize earlier response rather than perfect prevention.
- Ask patients to name more than one coping option for the same need.
- Normalize multiple recovery pathways and support preferences.

USEFUL PROBES

- Who can know before secrecy takes over?
- What need tends to get assigned to one coping strategy over and over?
- What second or third option could do part of the job?

MUST-LAND TEACHING

Clinical anchor: Self-monitoring, identifying needs, disclosure to trusted supports, expanding coping options, and continuing care can help and may reduce risk; they are not guarantees.

BOUNDARY / DO NOT OVERCLAIM

Guardrail: Do not imply one fellowship, support model, or coping strategy fits everyone.

STEPHANIE VOICE / OPTIONAL

"One coping skill doesn't need seventeen jobs" stays. It has earned tenure.

TRANSITION

Give them a compact way to operationalize this.

33. MY RECOVERY DASHBOARD

INSTRUCTIONAL PURPOSE

Convert the group into a simple personal monitoring plan.

WHAT THE PATIENT SEES

WATCH • NOTICE • NEED • TRY • TALK.

FACILITATION FLOW

- Treat this as a planning tool, not an assessment instrument.
- Have patients identify one pattern, one early warning sign, one underlying need, one alternative response, and one person/support.
- Encourage concrete answers small enough to use outside group.

USEFUL PROBES

- What would tell you early—not after the wheels are off—that something is changing?
- Who can you tell before you start hiding it?

MUST-LAND TEACHING

Clinical anchor: The dashboard supports self-monitoring and earlier response; it does not replace individualized assessment or safety planning.

BOUNDARY / DO NOT OVERCLAIM

Guardrail: Do not add crisis/medical instructions unless that is the program's normal handout practice.

TRANSITION

Return to the exact question that opened the group.

34. WHEN THE SUBSTANCE CHANGES, DOES THE PATTERN CHANGE?

INSTRUCTIONAL PURPOSE

Close by shifting recovery from stopping alone to building alternatives.

WHAT THE PATIENT SEES

Opening question; reveal: "Recovery isn't only about what I stop." → "It's also about what I build instead."

FACILITATION FLOW

- Ask the opening question again before revealing anything.
- Let patients answer using the language they developed during group.
- Reveal the two closing lines slowly.
- End there. No generic recap and no "Questions?" slide.

USEFUL PROBES

- What do you want to build that makes the old solution less necessary?

MUST-LAND TEACHING

Clinical anchor: Recovery-oriented care includes what a person builds, practices, and supports—not only what is removed.

BOUNDARY / DO NOT OVERCLAIM

Guardrail: Do not frame recurrence as failure; it signals a need to reassess support or treatment.

TRANSITION

Stop. The ending has already done its job.

Facilitator Reference: Parked Teaching Branches

Use these only when the discussion naturally calls for them. They are facilitator material, not additional slide requirements.

Brain/computer analogy

Use it to explain pattern learning: the brain notices what produces relief/reward and makes that response easier to retrieve. State that this is an analogy, not literal neuroscience. Avoid “virus” or “coding” language as if it were a biological mechanism.

Choice and consequence analogy

The teaching purpose is to separate choosing an exposure from choosing every later biological or behavioral response. Keep analogies limited; do not claim mechanistic equivalence between addiction and unrelated medical conditions.

Cannabis

Cannabis can involve tolerance, withdrawal, and cannabis use disorder. Current products can contain higher THC concentrations than older products. Do not turn population-level risk into an individual prediction or use generational one-upmanship.

Cold-turkey stories

A person’s successful cold-turkey experience does not establish medical safety for someone else. Alcohol and benzodiazepine withdrawal can be medically dangerous. Opioid withdrawal is usually not itself life-threatening, but loss of tolerance and return to use can increase overdose danger.

Kratom / 7-OH / retail products

Do not put kratom, concentrated 7-OH, tianeptine, intoxicating hemp products, cannabis, CBD, and prescribed medication into one bucket. Kratom is not FDA-approved to treat OUD, pain, anxiety, or withdrawal; concentrated 7-OH products warrant separate opioid-risk discussion.

CBD / intoxicating hemp

Do not use “CBD” as shorthand for every hemp-derived retail product. Contents and intoxicating potential vary; some delta-8 THC products can be synthesized from CBD.

Melatonin anecdote

If the story comes up, describe feeling unable to sleep without melatonin as possible psychological reliance unless broader evidence supports an addiction/disorder pattern.

Gas-station drugs

Ease of access and legal retail availability can influence exposure and perceived safety. “Sold at a gas station” is an access/context fact, not a clinical drug class.

Humor boundaries

Aim sarcasm at bad logic, outdated myths, or absurd overgeneralizations. Do not aim it at patients, MOUD, pain, withdrawal, trauma, sexuality, or a disclosure that cost someone courage to make.

Evidence Map

The facilitator guide follows the September 17, 2026 evidence audit. The source letters below match that audit's register; consult the audit for full source details and links.

Topic	Audit source letters
Cross-addiction terminology / substitution	L, O
SUD / treatment principles	A, B, V, W
Gateway/common liability	I, J, K
MOUD / OUD treatment	C, D, E
Alcohol withdrawal	F
Benzodiazepine withdrawal	G
Cannabis	H, AC
Behavioral diagnostic distinctions	M, N, O, P
Kratom / 7-OH	Q, R, S, T
CBD / hemp	U
Pain / opioid prescribing context	Z
Tolerance / dependence distinctions	AA
Reinforcement / learning	X, Y

Retired Legacy Material — Do Not Reintroduce

- "Addiction Interaction Disorder" as an alternate diagnostic name.
- "Two or more addictive behaviors" as the definition of cross-addiction.
- Causal soft-to-hard gateway teaching or the claim that "soft" and "hard" drugs cause the same brain reaction.
- "Soft" and "hard" as formal clinical drug classes.
- Suboxone/buprenorphine, kratom, marijuana, and CBD grouped together as "rescue drugs."
- Deterministic claims that cross-addiction inevitably leads back to the favorite substance or blocks recovery.
- The 2018 prevalence arithmetic and the "as many as 25%" substitution statistic.
- Treating all process behaviors as equivalent formal addiction diagnoses.
- Generalizing tolerance or withdrawal to every repetitive behavior.
- A single-cause recipe of genetics + society + trauma + family strain + mental illness.

Facilitator Closing Note

The point of the group: Help patients notice when a familiar function, loss of freedom, narrowing focus, or growing fallout is reappearing in a new form—without forcing every pattern into an addiction label.

Recovery isn’t only about what I stop. It’s also about what I build instead.